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/ **Practice Exam - 1**

Correct Answer

Next Question

Practice Exam - 1

Question 105 of 117



A 29-year-old female with no significant past medical history presents with gradually progressive dyspnea on exertion over the past two years. She has dyspnea climbing a flight of stairs or walking up inclines, but she can perform activities of daily living without difficulty. Her paternal aunt died at a young age, and post-mortem genetic testing showed a pathologic variant in the BMPR2 gene. Vitals are within normal limits. A complete metabolic panel, complete blood count, and B-type natriuretic peptide level are normal. ECG shows sinus rhythm 68 bpm. She walks 450 meters in 6 minutes. She undergoes right heart catheterization, with results shown below:

Baseline: RA 5 mmHg, PA 65/35 (45) mmHg, PCW 12 mmHg, CO 5.1 L/min, PVR 6.5 Wood units Nitric oxide (20 ppm for 5 minutes): RA 4 mmHg, PA 50/24 (33) mmHg, PCW 10 mmHg, CO 5.3 L/min, PVR 4.3 Wood units

What would be the next most appropriate treatment?

A. Ambrisentan plus tadalafil



B. Intravenous treprostinil



C. Nifedipine

D. Verapamil

Commentary References

Testing Point: The Board-certified AHFTC specialist should know the appropriate treatment for group 1 pulmonary hypertension.

Answer: C. Nifedipine

Distractors: The minimally-qualified AHFTC specialist might select another answer... This patient has group 1 pulmonary hypertension, WHO functional class II symptoms, and a positive vasoreactivity test (Mean PA pressure decreases by ≥ 10 mmHg to an absolute value ≤ 40 mmHg with unchanged or increased cardiac output). Calcium channel blockers such as nifedipine, amlodipine, and diltiazem are recommended treatment options.

Answer A: Combination therapy is appropriate for patients without vasoreactivity or those with vasoreactivity who do not improve on a calcium channel blocker. This patient had a positive vasoreactivity study, and is treatment naïve.

Answer B: Parenteral or inhaled prostacyclins are not recommended as initial therapy for WHO functional class II symptoms.

Answer D: Verapamil has significant negative inotropy and should generally be avoided in pulmonary hypertension.

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